

Policy Title:	B.1.1. MAINTAINING HIGH PROFESSIONAL STANDARDS IN THE MODERN NHS: Conduct, Capability and Ill-Health, Policies and Procedures for Medical & Dental Practitioners		
Executive Summary:	This policy implements the national policy framework ‘Maintaining High Professional Standards in the Modern NHS’. In implementing this policy framework, the Trust is acting in accordance with national guidelines for matters relating to discipline and exclusion of medical staff.		
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MAINTAINING HIGH PROFESSIONAL STANDARDS IN THE MODERN NHS:

Conduct, Capability and Ill-Health, Policies and Procedures for Medical & Dental Practitioners

1.0 INTRODUCTION

This policy implements the national policy framework 'Maintaining High Professional Standards in the Modern NHS'. In implementing this policy framework, the Trust is acting in accordance with Directions on Disciplinary Procedures 2005 and the Restriction of Practice and Exclusion from Work Directions 2003. This Trust policy document incorporates the national framework by reference. The national framework in full is as published on the Department of Health's website

These Disciplinary arrangements replaced the disciplinary procedures enshrined in HC(90)9. The arrangements also replace the provisions in HC(82)13 for Special Professional Panels (the "Three Wise Men"). The right of appeal to the Secretary of State held by certain practitioners under paragraph 190 of their terms and conditions of service is lost. This Policy also replaces the previous Trust Disciplinary Procedures for Medical and Dental Staff.

This Trust policy document mirrors the national framework for dealing with disciplinary issues for medical and dental staff.

The framework comprises five parts:

- I Action when a concern arises
- II Restriction of practice and exclusion
- III Conduct hearings and disciplinary matters
- IV Procedures for dealing with issues of capability
- V Handling concerns about a practitioner's health

The Trust will act in accordance with the national framework from which this policy document is drawn when concerns arise.

In this spirit of the framework, the Trust undertakes to ensure that through any investigation be it formal or informal, full account will always be taken of

considerations of the health of the practitioner, system failure and working environment. It will seek where possible to tackle performance issues through training or other remedial action rather than solely through disciplinary action. It will continue to support medical and dental staff in their professional development in particular through appraisal.

2.0 SCOPE

The framework applies to all doctors and dentists employed by the Trust. This policy applies to all medical and dental staff employed by the Trust irrespective of grade or department.

3.0 RESPONSIBILITIES

The responsibilities of officers of the Trust are detailed in Sections 10.1 to 10.5.

4.0 CONTEXT

There is no distinction between personal and professional misconduct and doctors and dentists will be disciplined under the same locally-based procedures as any other member of staff.

The processes for conducting investigations (and where necessary the steps necessary to exclude a member of the medical and dental staff) when a concern arises, are mapped out in Sections 10.1, 10.2 and 10.3. There is scope to bring in expert advice for panels considering capability issues as shown in Section 10.4. Health issues should be routinely dealt with through the Occupational Health service (Section 10.5) and the Trust encourages alternatives to be found to disciplinary action when dealing with individuals with health problems.

5.0 APPROVAL AND RATIFICATION PROCESS

This Trust policy document has been agreed with the Local Negotiating Committee, as will any future versions amended in accordance with changes to the national framework.

6.0 REVIEW AND REVISION ARRANGEMENTS

This Trust policy document may be amended from time to time to reflect changes in the national policy framework. Where there is any procedural dispute the first point of reference will be the national policy framework from which this Trust policy document is drawn.

7.0 DISSEMINATION AND IMPLEMENTATION

This Policy will be placed on the Trust Intranet and notification sent to all Medical and Dental staff that it can be found on that site. All new starters will be informed that their appointment is subject to this policy.

8.0 MONITORING COMPLIANCE AND EFFECTIVENESS

Disciplinary cases will be reported (for medical and non-medical staff) together. An annual report will be produced by the Director of Human Resources for the HR Committee and LNC

9.0 REFERENCES AND ASSOCIATED DOCUMENTATION

Maintaining High Professional Standards in the Modern NHS: Doctors' and dentists' disciplinary framework, Department of Health, 15 February 2005.

Directions on Disciplinary Procedures, Department of Health, 17 February 2005.

Restriction of Practice and Exclusion from Work Directions 2003, Department of Health, 29 December 2003.

HC(82)13 for Special Professional Panels (the "Three Wise Men"), Department of Health Circular, 1982.

European Convention on Human Rights, Council of Europe, European Treaties.

10.0 POLICY FRAMEWORK

The framework comprises five parts:

- I Action when a concern arises**
- II Restriction of practice and exclusion**
- III Conduct hearings and disciplinary matters**
- IV Procedures for dealing with issues of capability**

V Handling concerns about a practitioner's health**10.1 PART I****ACTION WHEN A CONCERN ARISES****INTRODUCTION**

1. The management of performance is a continuous process which is intended to identify problems. Numerous ways now exist in which concerns about a practitioner's performance can be identified; through which remedial and supportive action can be quickly taken before problems become serious or patients harmed; and which need not necessarily require formal investigation or the resort to disciplinary procedures.

Concerns about a doctor or dentist's conduct or capability can come to light in a wide variety of ways, for example:

- Concerns expressed by other NHS professionals, health care managers, students and non-clinical staff
 - Review of performance against job plans, annual appraisal, revalidation
 - Monitoring of data on performance and quality of care
 - Clinical governance, clinical audit and other quality improvement activities
 - Complaints about care by patients or relatives of patients
 - Information from the regulatory bodies
 - Litigation following allegations of negligence
 - Information from the police or coroner
 - Court judgements
2. Unfounded and malicious allegations can cause lasting damage to a doctor's reputation and career prospects. Therefore, all allegations, including those made by relatives of patients, or concerns raised by colleagues, must be properly investigated to verify the facts so that the allegations can be shown to be true or false.

FRAMEWORK FOR NHS PROCEDURES

3. The Trust has procedures for handling serious concerns about an individual's conduct and capability. These procedures reflect the framework in this document and allow for informal resolution of less serious problems. Concerns about the capability of doctors and dentists in training will be

considered initially as training issues and the Postgraduate Dean will be involved from the outset. Informal resolution may include the clinical manager providing the practitioner with additional support/supervision, setting targets and objectives and measuring these or issuing a letter warning the practitioner of formal procedures that could follow if their conduct is an issue. Informal processes should always be documented and the practitioner should have a summary of discussions that have taken place and actions that have been agreed.

4. All serious concerns will be registered with the Chief Executive and he or she will ensure that a case manager is appointed. The Chairman of the Board will designate a non-executive member “the designated member” to oversee the case and ensure that momentum is maintained. All concerns should be investigated quickly and appropriately. A clear audit route will be established for initiating and tracking progress of the investigation, its costs and resulting action. However, the issue is raised, the Medical Director will work with the Director of HR to decide the appropriate course of action in each case. A Decision Making Group will be convened to agree appropriate action where necessary. The Terms of Reference for this group can be found in Appendix 1. The Medical Director will act as the case manager in cases involving Chief of Divisions and may delegate this role to Clinical Leads or a senior manager to oversee the case on his or her behalf in other cases. The Medical Director is responsible for appointing a case investigator.

Protecting the Public

5. When serious concerns are raised about a practitioner, the Trust will urgently consider whether it is necessary to place temporary restrictions on their practice. This might be to amend or restrict their clinical duties, obtain undertakings or provide for the exclusion of the practitioner from the workplace. Part II of this framework sets out the procedures for this action.
6. The duty to protect patients is paramount. At any point in the process where the case manager has reached the clear judgement that a practitioner is considered to be a serious potential danger to patients or staff, that practitioner must be referred to the regulatory body, whether or not the case has been referred to the Practitioner Performance Advisory Service (PPAS). Consideration should also be given to whether the issue of an alert letter should be requested. The GMC or GDC will discuss with the Practitioner Performance Advisory Service (PPAS) whether any immediate action is needed by the GMC/GDC or whether the Practitioner Performance Advisory Service (PPAS)’s consideration should continue.

Involving the Practitioner Performance Advisory Service (PPAS)

7. At any stage the handling of a case consideration should be given to the involvement of the Practitioner Performance Advisory Service (PPAS). The Practitioner Performance Advisory Service (PPAS) has developed a staged approach to the services it provides NHS Trusts and practitioners. This involves:

- Immediate telephone advice, available 24 hours
- Advice, then detailed supported local case management
- Advice, then supported local clinical performance assessment
- Advice, then detailed Practitioner Performance Advisory Service (PPAS) clinical performance assessment
- Support with implementation of recommendations arising from assessment
- Understanding the issue and investigation.

Summary of key action

- Clarify what has happened and the nature of the problem or concern;
 - Discuss with the Practitioner Performance Advisory Service (PPAS) what the way forward should be;
 - Consider whether restriction of practice or exclusion is required;
 - If a formal approach under the conduct or capability procedures is required, appoint an investigator;
 - If the case can be progressed by agreement, consider whether an Practitioner Performance Advisory Service (PPAS) assessment would help clarify the underlying factors that led to the concerns and assist with identifying the solution.
8. The first task of the case manager is to identify the nature of the problem or concern and to assess the seriousness of the issue on the information available and the likelihood that it can be resolved without resort to formal disciplinary procedures. This is a difficult decision and will not be taken alone, but in consultation with the Director of HR and the Medical Director and the National Clinical Assessment Service (Practitioner Performance Advisory Service). The Practitioner Performance Advisory Service (PPAS) can provide a sounding board for the case manager's first thoughts. However, the Practitioner Performance Advisory Service (PPAS) asks that the first approach to them should be made by the Trust's Chief Executive or Medical Director. Where there are concerns about a doctor or dentist in training, the Postgraduate Dean will be involved as soon as possible.

9. The first stage of the Practitioner Performance Advisory Service (PPAS)'s involvement in a case is exploratory – an opportunity for local managers to discuss the problem with an impartial outsider, to look afresh at a problem, see new ways of tackling it themselves, possibly recognise the problem as being more to do with work systems than doctor performance, or see a wider problem needing the involvement of an outside body other than the Practitioner Performance Advisory Service (PPAS).
10. Having discussed the case with the Practitioner Performance Advisory Service (PPAS), the case manager will decide whether an informal approach can be taken to address the problem, or whether a formal investigation will be needed. Where an informal route is chosen the Practitioner Performance Advisory Service (PPAS) can still be involved until the problem is resolved. This can include the Practitioner Performance Advisory Service (PPAS) undertaking a formal clinical performance assessment when the doctor, the NHS body and the Practitioner Performance Advisory Service (PPAS) agree that this could be helpful in identifying the underlying cause of the problem and possibly remedial steps. If the Practitioner Performance Advisory Service (PPAS) is asked to undertake an assessment of the doctor's practice, the outcome of a local investigation may be made available to inform the Practitioner Performance Advisory Service (PPAS)'s work.
11. Where it is decided that a more formal route needs to be followed (perhaps leading to conduct or capability proceedings) the Medical Director will, after discussion between the Chief Executive and Director of Human Resources, appoint an appropriately experienced or trained person as case investigator. The seniority of the case investigator will differ depending on the grade of practitioner involved in the allegation. Several clinical managers should be appropriately trained, to enable them to carry out this role when required.
12. The case investigator is responsible for leading the investigation into any allegations or concerns about a practitioner, establishing the facts and report the findings. The case investigator:
 - Must formally involve a senior member of the medical or dental staff where a question of clinical judgement is raised during the investigation process.
 - Must ensure that safeguards are in place throughout the investigation so that breaches of confidentiality are avoided as far as possible. Patient confidentiality needs to be maintained, but the disciplinary panel will need to know the details of the allegations. It is the responsibility of the case investigator to judge what information needs to be gathered and how – within the boundaries of the law – that information should be gathered.

- Must ensure that there are sufficient written statements collected to establish a case prior to a decision to convene a disciplinary panel and on aspects of the case not covered by a written statement, ensure that oral evidence is given sufficient weight in the investigation report.
- Must ensure that a written record is kept of the investigation, the conclusions reached and the course of action agreed by the Director of HR and with the Medical Director
- Must assist the designated Board member in reviewing the progress of the case.

The case investigator does not make the decision on what action should be taken nor whether the employee should be excluded from work and may not be a member of any disciplinary or appeal panel relating to the case.

13. The practitioner concerned must be informed in writing by the case manager, as soon as it has been decided, that an investigation is to be undertaken, the name of the case investigator and made aware of the specific allegations or concerns that have been raised. The practitioner must be given the opportunity to see any correspondence relating to the case together with a list of the people that the case investigator will interview. The practitioner must also be afforded the opportunity to put their view of events to the case investigator and given the opportunity to be accompanied.
14. At any stage of this process – or subsequent disciplinary action – the practitioner may be accompanied in any interview or hearing by a companion. In addition to statutory rights under the Employment Act 1999, the companion may be another employee of the NHS body; an official or lay representative of the British Medical Association, British Dental Association or defence organisation; or a friend, partner or spouse. The companion may be legally qualified, but he or she will not normally be acting in a legal capacity.

Requests to be accompanied by a legal representative will be considered by the case investigator or Director of HR on a case by case basis. Where the allegations against an employee are serious and dismissal may be an outcome, particularly to the extent that if dismissed the employee may lose any future prospect of employment in his/her profession due to loss or restriction of membership with his/her professional body, it may be considered reasonable to allow an employee legal representation. Requests should be submitted in writing to the Director of HR and a written response should be given within 5 working days of receipt of the request.

15. The case investigator has wide discretion on how the investigation is carried out, but in all cases the purpose of the investigation is to ascertain the facts

in an unbiased manner. Investigations are not intended to secure evidence against the practitioner as information gathered in the course of an investigation may clearly exonerate the practitioner or provide a sound basis for effective resolution of the matter.

16. If during the source of the investigation it transpires that the case involves more complex clinical issues than first anticipated, the case manager should consider whether an independent practitioner from another NHS body should be invited to assist.
17. The case investigator should complete the investigation within 4 weeks of appointment and submit their report to the case manager within a further 5 days. The report of the investigation should give the case manager sufficient information to make a decision whether:
 - There is a case of misconduct that should be put to a conduct panel;
 - There are concerns about the practitioner's health that should be considered by the NHS body's occupational health service;
 - There are concerns about the practitioner's performance that should be further explored by the National Clinical Assessment Service;
 - Restrictions on practice or exclusion from work should be considered;
 - There are serious concerns that should be referred to the GMC or GDC;
 - There are intractable problems and the matter should be put before a capability panel;
 - No further action is needed.

Involvement of the Practitioner Performance Advisory Service (PPAS) following local investigation

18. Medical underperformance can be due to health problems, difficulties in the work environment, behaviour or a lack of clinical capability. These may occur in isolation or in a combination. The Practitioner Performance Advisory Service (PPAS)'s processes are aimed at addressing all of these, particularly where local action has not been able to take matters forward successfully. The Practitioner Performance Advisory Service (PPAS)'s methods of working therefore assume commitment by all parties to take part constructively in a referral to the Practitioner Performance Advisory Service (PPAS). For example, its assessors work to formal terms of reference, decided on after input from the doctor and the referring body.

19. The focus of the Practitioner Performance Advisory Service (PPAS)'s work is therefore likely to involve performance difficulties which are serious and/or repetitive. That means:
- Performance falling well short of what doctors and dentists could be expected to do in similar circumstances and which, if repeated, would put patients seriously at risk.
 - Alternatively or additionally, problems that are ongoing or (depending on severity) have been encountered on at least two occasions.
 - In cases where it becomes clear that the matters at issue focus on fraud, specific patient complaints or organisational governance, their further management may warrant a different local process. The Practitioner Performance Advisory Service (PPAS) may advise on this.
20. Where an employing body is considering excluding a doctor or dentist whether or not his or her performance is under discussion with the Practitioner Performance Advisory Service (PPAS), it is important for the Practitioner Performance Advisory Service (PPAS) to know of this at an early stage, so that alternatives to exclusion can be considered. Procedures for exclusion are covered in part II of the framework. It is particularly desirable to find an alternative when the Practitioner Performance Advisory Service (PPAS) is likely to be involved, because it is much more difficult to assess a doctor who is excluded from practice than one who is working.
21. A practitioner undergoing assessment by the Practitioner Performance Advisory Service (PPAS) must co-operate with any request to give an undertaking not to practise in the NHS or private sector other than their main place of NHS employment until the Practitioner Performance Advisory Service (PPAS) assessment is complete.
22. Failure to co-operate with a referral to the Practitioner Performance Advisory Service (PPAS) may be seen as evidence of a lack of willingness on the part of the doctor or dentist to work with the employer on resolving performance difficulties. If the practitioner chooses not to co-operate with such a referral, that may limit the options open to the parties and may necessitate disciplinary action and consideration of referral to the GMC or GDC.

Confidentiality

23. The Trust will always maintain confidentiality. No press notice should be issued, nor the name of the practitioner released, regarding any investigation or hearing into disciplinary matters. The Trust will only confirm that an investigation or disciplinary hearing is underway.

24. Personal data released to the case investigator for the purposes of the investigation must be fit for the purpose, nor disproportionate to the seriousness of the matter under investigation. The guiding principles of the Data Protection Act will be followed.

10.2 PART II**RESTRICTION OF PRACTICE AND EXCLUSION FROM WORK****INTRODUCTION**

1. This part of the framework replaced the guidance in HSG(94)49 – Disciplinary Procedures for Hospital and Community Medical and Hospital Dental Staff. Department of Health, 1994 and should be read in conjunction with the Trust's Disciplinary Policy and Procedure, HR policy B.1.A

Managing the Risk to Patients

2. When serious concerns are raised about a practitioner, the Trust will urgently consider whether it is necessary to place temporary restrictions on their practice. This might be to amend or restrict their clinical duties, obtain undertakings or provide for the exclusion of the practitioner from the workplace. Where there are concerns about a doctor or dentist in training, the Postgraduate Dean should be involved as soon as possible.

Exclusion of clinical staff from the workplace is a temporary expedient. Under this framework, exclusion is a precautionary measure and not a disciplinary sanction. Exclusion from work ("suspension") should be reserved for only the most exceptional circumstances.

3. The purpose of exclusion is:
 - To protect the interests of patients or other staff; and/or
 - To assist the investigative process when there is a clear risk that the practitioner's presence would impede the gathering of evidence.

It is imperative that exclusion from work is not misused or seen as the only course of action that could be taken. The degree of action must depend on the nature and seriousness on the concerns and on the need to protect patients, the practitioner concerned and/or their colleagues.

4. Alternative ways to manage risks, avoiding exclusion, include:
 - Medical Director, Deputy Medical Director or Clinical Chief of Division supervision of normal contractual clinical duties.
 - Restricting the practitioner to certain forms of clinical duties.

- Restricting activities to administrative, research/audit, teaching and other educational duties. By mutual agreement the latter might include some formal retraining or re-skilling.
 - Sick leave for investigation of specific health problems.
5. In cases relating to the capability of a practitioner, consideration should be given to whether an action plan to resolve the problem can be agreed with the practitioner. Advice on the practicality of this approach should be sought from the National Clinical Assessment Service (Practitioner Performance Advisory Service (PPAS)). If the nature of the problem and a workable remedy cannot be determined in this way, the case manager should seek to agree with the practitioner to refer the case to the Practitioner Performance Advisory Service (PPAS), which can assess the problem in more depth and give advice on any action necessary. The Practitioner Performance Advisory Service (PPAS) can offer immediate telephone advice to case managers considering restriction of practise or exclusion and, whether the practitioner is excluded, provide an analysis of the situation and offer advice to the case manager.

THE EXCLUSION PROCESS

6. The Trust cannot require the exclusion of a practitioner for more than four weeks at a time. The justification for continued exclusion must be reviewed on a regular basis and before any further four week period of exclusion is imposed. Under the framework key officers and the Board have responsibilities for ensuring that the process is carried out quickly and fairly, kept under review and that the total period of exclusion is not prolonged.

Key Features of Exclusion from Work

- An initial “immediate” exclusion of no more than two weeks if warranted.
- Notification of the Practitioner Performance Advisory Service (PPAS) before formal exclusion.
- Formal exclusion (if necessary) for periods up to four weeks.
- Advice on the case management plan from the Practitioner Performance Advisory Service (PPAS).
- Appointment of a Board member to monitor the exclusion and subsequent action.
- Referral to Practitioner Performance Advisory Service (PPAS) for formal assessment, if part of case management plan.
- Active review to decide renewal or cessation of exclusion.
- A right to return to work if review not carried out.
- Performance reporting on the management of the case.

- Programme for return to work if not referred to disciplinary procedures or performance assessment.

Roles of Officers

7. The Chief Executive has overall responsibility for managing exclusion procedures and for ensuring that cases are properly managed. The decision to exclude a practitioner will normally be made by a Decision Making Group (detailed in Appendix 1). The case should be fully discussed with the Chief Executive, the Medical Director, the Director of Human Resources, the Practitioner Performance Advisory Service (PPAS) and other interested parties (such as the Police where there are serious criminal allegations or the Counter Fraud & Security Management Service) prior to the decision to exclude a practitioner. In the rare cases where immediate exclusion is required, the above parties must discuss the case at the earliest opportunity following exclusion, preferably at a Decision Making Group.
8. The authority to exclude a member of staff must be vested in a nominated manager or managers of the Trust. These managers should be at an appropriate senior level in the organisation. It should include the Chief Executive, Medical Director, Deputy Medical Director and the Clinical Chief of Division for staff below the grade of consultant.
9. The Medical Director will act as the case manager or delegate the role to a senior manager to oversee the case and appoint a case investigator to explore and report on the circumstances that have led to the need to exclude the staff member. The investigating officer will provide factual information to assist the case manager in reviewing the need for exclusion and making reports on progress to the Chief Executive or designated Board member

Role of Designated Board Member

10. Representations may be made to the designated Board member regarding exclusion, or investigation of a case if these are not provided for by the NHS body's grievance procedures. The designated Board member must also ensure, among other matters, that time frames for investigation or exclusion are consistent with the principles of Article 6 of the European Convention on Human Rights (which, broadly speaking, sets out the framework of the rights to a fair trial).

Immediate Exclusion

11. An immediate time limited exclusion may be necessary for the purposes identified in paragraph 3 above following:

- A critical incident when serious allegations have been made; or
- There has been a breakdown in relationships between a colleague and the rest of the team; or
- The presence of the practitioner is likely to hinder the investigation.

Such an exclusion will allow a more measured consideration to be undertaken. This period should be used to carry out a preliminary situation analysis, to contact the Practitioner Performance Advisory Service (PPAS) for advice and to convene a Decision Making Group. The manager making the exclusion must explain why the exclusion is being made in broad terms (there may be no formal allegation at this stage) and agree a date up to a maximum of two weeks away at which the practitioner should return to the workplace for a further meeting. The case manager must advise the practitioner of their rights, including rights of representation.

Formal Exclusion

12. A formal exclusion may only take place after the case manager has first considered whether there is a case to answer and then considered, at a Decision Making Group, whether there is reasonable and proper cause to exclude. The Practitioner Performance Advisory Service (PPAS) must be consulted where formal exclusion is being considered. If a case investigator has been appointed he or she must produce a preliminary report as soon as is possible to be available for the Decision Making Group. This preliminary report is advisory to enable the case manager to decide on the steps as appropriate.
13. The report should provide sufficient information for a decision to be made as to whether:
 - The allegation appears unfounded; or
 - There is a misconduct issue; or
 - There is a concern about the practitioner's capability; or
 - The complexity of the case warrants further detailed investigation before advice can be given on the way forward and what needs to be inquired into.
14. Formal exclusion of one or more clinicians must only be used where:
 - a) There is a need to protect the interests of patients or other staff pending the outcome of a full investigation of:

- Allegations of misconduct.
- Concerns about serious dysfunctions in the operation of a clinical service.
- Concerns about lack of capability or poor performance of sufficient seriousness as to justify formal exclusion;

or

- b) The presence of the practitioner in the workplace is likely to hinder the investigation.
15. Full consideration should be given to whether the practitioner could continue in or (in cases of an immediate exclusion) return to work in a limited capacity or in an alternative, possibly non-clinical role, pending the resolution of the case.
16. When the practitioner is informed of the exclusion, there should, where practical, be a witness present and the nature of the allegations or areas of concern should be conveyed to the practitioner. The practitioner should be told of the reason(s) why formal exclusion is regarded as the only way to deal with the case. At this stage the practitioner should be given the opportunity to state their case and propose alternatives to exclusion (e.g. further training, referral to occupational health, referral to the Practitioner Performance Advisory Service (PPAS) with voluntary restriction).
17. The formal exclusion must be confirmed in writing as soon as is reasonably practicable. The letter should state the effective date and time, duration (up to 4 weeks), the content of the allegations, the terms of the exclusion (e.g. exclusion from the premises, see paragraph 21, and the need to remain available for work paragraph 22) and that a full investigation or what other action will follow. The practitioner and their companion should be advised that they may make representations about the exclusion to the designated Board member at any time after receipt of the letter confirming the exclusion.
18. In cases when disciplinary procedures are being followed, exclusion may be extended for four week renewable periods until the completion of disciplinary procedures if a return to work is considered inappropriate. The exclusion should still only last for four weeks at a time and be subject to review. The exclusion should usually be lifted and the practitioner allowed back to work, with or without conditions placed upon the employment, as soon as the original reasons for exclusion no longer apply.
19. If the case manager considers that the exclusion will need to be extended over a prolonged period outside of his or her control (for example because of a police investigation), the case must be referred to the Practitioner

Performance Advisory Service (PPAS) for advice as to whether the case is being handled in the most effective way and suggestions as to possible ways forward. However, even during this prolonged period the principle of four week “renewability” must be adhered to.

20. If at any time after the practitioner has been excluded from work, investigation reveals that either the allegations are without foundation or that further investigation can continue with the practitioner working normally or with restrictions, the case manager must lift the exclusion and make arrangements for the practitioner to return to work with any appropriate support as soon as practicable.

Exclusion from Premises

21. Practitioners should not be automatically barred from the premises upon exclusion from work. Case managers must always consider whether a bar from the premises is absolutely necessary. There are certain circumstances, however, where the practitioner should be excluded from the premises. This could be, for example, where there may be a danger of tampering with evidence, or where the practitioner may be a serious potential danger to patients to exclude the practitioner from the premises. The practitioner may want to retain contact with colleagues, take place in clinical audit and to remain up to date with developments in their field of practice or to undertake research or training.

Keeping in Contact and Availability for Work

22. As exclusion under this framework should usually be on full pay, the practitioner must remain available for work with their employer during their normal contracted hours. The practitioner must inform the case manager of any other organisation(s) with whom they undertake either voluntary or paid work and seek their case manager’s consent to continuing to undertake such work or to take annual leave or study leave. The practitioner should be reminded of these contractual obligations but would be given 24 hours notice to return to work. In exceptional circumstances the case manager may decide that payments are not justified because the practitioner is no longer available for work (e.g. abroad without agreement).

The case manager should make arrangements to ensure that the practitioner can keep in contact with colleagues on professional developments, and take part in Continuing Professional development (CPD) and clinical audit activities with the same level of support as other doctors or dentists in their employment. A mentor could be appointed for the purpose if a colleague is willing to undertake this role.

Informing Other Organisations

23. In cases where there is concern that the practitioner may be a danger to patients, the employer has an obligation to inform such other organisations including the private sector, of any restriction on practice or exclusion and provide a summary of the reasons for it. Details of other employers (NHS and non-NHS) may be readily available from job plans, but where it is not the practitioner should supply them. Failure to do so may result in further disciplinary action or referral to the relevant regulatory body, as the paramount interest is the safety of patients. Where an NHS employer has placed restrictions on practice, the practitioner should agree not to undertake any work in that area of practice with any other employer.
24. Where the case manager believes that the practitioner is practising in other parts of the NHS or in the private sector in breach of defiance of an undertaking not to do so, he or she should contact the professional regulatory body and the Practitioner Performance Advisory Service (PPAS) to consider the issue of an alert letter.

Informal Exclusion

25. No practitioner should be excluded from work other than through this new procedure.

KEEPING EXCLUSIONS UNDER REVIEW

Informing the Board

26. The Board must be informed about an exclusion at the earliest opportunity. The Board has a responsibility to ensure that the organisation's internal procedures are being followed. It should, therefore:
- Require a summary of the progress of each case at the end of each period of exclusion, demonstrating that procedures are being correctly followed and that all reasonable efforts are being made to bring that situation to an end as quickly as possible.
 - Receive a monthly statistical summary showing all exclusions with their duration and number of times the exclusion had been reviewed and extended.

Regular Review

27. The case manager must review the exclusion before the end of each four week period and report the outcome to the Chief Executive and the Board. This report is advisory and it would be for the case manager to decide on the next steps as appropriate. The exclusion should usually be listed and the practitioner allowed back to work, with or without conditions placed upon the employment, at any time the original reasons for exclusion no longer apply and there are no other reasons for exclusion. The exclusion will lapse and the practitioner will be entitled to return to work at the end of the four-week period if the exclusion is not actively reviewed.
28. The Trust must take review action before the end of each 4 week period. This will be done at a Decision Making Group. After three exclusions, the Practitioner Performance Advisory Service (PPAS) must be called in. The table below outlines the various activities that must be undertaken at different stages of exclusion.

First and Second Interviews (and reviews after the third week)

Before the end of each exclusion (of up to 4 weeks) the case manager reviews the position.

- The case manager decides on next steps as appropriate. Further renewal may be for up to 4 weeks at a time.
- Case manager submits advisory report of outcome to Chief Executive and the Board.
- Each renewal is a formal matter and must be documented as such.
- The practitioner must be sent written notification on each occasion.
- Third review.

If the practitioner has been excluded for three periods:

- A report must be made to the Chief Executive outlining the reasons for the continued exclusion and why restrictions on practice would not be an appropriate alternative; and if the investigation has not been completed a timetable for completion of the investigation.
- The case must formally be referred to the National Clinical Assessment Service (Practitioner Performance Advisory Service (PPAS)) explaining:
 - Why continued exclusion is appropriate
 - What steps are being taken to conclude the exclusion at the earliest opportunity.

- The Practitioner Performance Advisory Service (PPAS) will review the case and advise the NHS body on the handling of the case until it is concluded.
29. Normally there should be a maximum limit of 6 months exclusion, except for those cases involving criminal investigations of the practitioner concerned. The employer and the Practitioner Performance Advisory Service (PPAS) should actively review those cases at least every six months.

The Role of the Board and Designated Member

33. The Board has a responsibility for ensuring that the procedures are established and followed. It is also responsible for ensuring the proper corporate governance of the organisation and for this purpose reports must be made to the Board under these procedures.
34. Board members may be required to sit as members of a disciplinary or appeal panel. Therefore, information given to the Board should only be sufficient to enable the Board to satisfy itself that the procedures are being followed. An alternative approach is to exclude 3 members at the outset from discussion of the case. Only the designated Board member should be involved to any significant degree in each review.
35. The Board is responsible for designating one of its non-executive members as a “designated Board member” under these procedures. The designated Board member is the person who oversees the case manager and investigating manager during the investigation process and maintains momentum of the process.
36. This member’s responsibilities include:
- Receiving reports and reviewing the continued exclusion from work of the practitioner.
 - Considering any representations from the practitioner about his or her exclusion.
 - Considering any representations about the investigation.

RETURN TO WORK

37. If it is decided that the exclusion should come to an end, there must be formal arrangements for the return to work of the practitioner. It must be clear whether clinical and other responsibilities are to remain unchanged or what the duties and restrictions are to be and any monitoring arrangements to ensure patient safety.

10.3 PART III**CONDUCT HEARINGS AND DISCIPLINARY MATTERS****INTRODUCTION**

1. All issues regarding the misconduct of doctors and dentists, as for all other staff, should be dealt with under the Trust's Disciplinary Policy and Procedure (Policy B.1.A). The Trust will nevertheless seek advice from the Practitioner Performance Advisory Service (PPAS) in conduct cases, particularly in cases of professional conduct, before proceeding.
2. Where the alleged misconduct relates to matters of a professional nature, or where an investigation identifies issues of professional conduct, the case investigator must obtain appropriate independent professional advice. Similarly, where a case involving issues of professional conduct proceeds to a hearing under the Trust's conduct procedures the panel must include a member who is medically qualified (in the case of doctors) or dentally qualified (in the case of dentists) and who is not currently employed by the organisation.

Codes of Conduct

3. The Trust has an agreed Disciplinary Policy and Procedure (Policy B.1.A), which sets out acceptable standards of conduct and behaviour expected of all its employees. Breaches of these rules are considered to be "misconduct". Misconduct can cover a very wide range of behaviour and can be classified in a number of ways, but it will generally fall into one or four distinct categories:
 - A refusal to comply with reasonable requirements of the employer.
 - An infringement of the employer's disciplinary rules including conduct that contravenes the standard of professional behaviour required by doctors and dentists by their regulatory body.
 - The commission of criminal offences outside the place of work which may, in particular circumstances, amount to misconduct.
 - Wilful, careless, inappropriate or unethical behaviour likely to compromise standards of care or patient safety or create serious dysfunction to the effective running of a service.
4. Examples of misconduct will vary greatly. The Trust's Disciplinary Policy and Procedure sets out details of some of the acts that will result in a serious breach of contractual terms and will constitute gross misconduct and could lead to

summary dismissal. The procedures cannot cover every eventuality. Similarly, the ACAS Code of Practice provides a non-exhaustive list of examples. Acts of misconduct may be simple and readily recognised or more involved. Examples include unreasonable or inappropriate behaviour such as verbal or physical bullying, harassment and/or discrimination in the exercise of their duties towards patients, the public or other employees. It also includes actions such as deliberate falsification or fraud.

5. Any allegation of misconduct against a doctor or dentist in recognised training grades will be considered initially as a training issue and dealt with via the educational supervisor and college or clinical tutor with close involvement of the Postgraduate Dean from the outset.
6. Failure to fulfil contractual obligations may also constitute misconduct. For example, regular non-attendance at clinics or ward rounds, or not taking part in clinical governance activities may come into this category. Additionally, instances of failing to give proper support to other members of staff including doctors or dentists in training may be considered in this category.
7. Each case must be investigated, but as a general rule no employee should be dismissed for a first offence, unless it is one of gross misconduct.
8. The Trust will decide upon the most appropriate way forward, having consulted the Practitioner Performance Advisory Service (PPAS) and its own employment law specialist. If a practitioner considers that the case has been wrongly classified as misconduct, he or she (or his/her representative) is entitled to use the employer's grievance procedure. Alternatively, or in addition he or she may make representations to the designated Board member.

ALLEGATIONS OF CRIMINAL ACTS

Action when investigations identify possible criminal acts

9. Where the Trust's investigation establishes a suspected criminal action in the UK or abroad, this must be reported to the police. The Trust investigation should only proceed in respect of those aspects of the case which are not directly related to the police investigation underway. The employer must consult the police to establish whether an investigation into any other matters would impede their investigation. In cases of fraud, the Counter Fraud & Security Management Service must be contacted.

Cases where criminal charges are brought not connected with an investigation by an NHS employer

10. There are some criminal offences that, if proven, could render a doctor or dentist unsuitable for employment. In all cases the Trust, having considered the facts, will need to consider whether the employee poses a risk to patients or colleagues and whether their conduct warrants instigating an investigation and the exclusion of the practitioner. The employer will have to give serious consideration to whether the employee can continue in their job once criminal charges have been made.

Bearing in mind the presumption of innocence, the Trust must consider whether the offence, if proven, is one that makes the doctor or dentist unsuitable for their type of work and whether, pending the trial, the employee can continue in their present job, should be allocated to other duties or should be excluded from work. This will depend on the nature of the offence and advice should be sought from an HR Adviser. The Trust will, as a matter of good practice, explain the reasons for taking such action.

Dropping of charges or no court conviction

11. When the Trust has refrained from acting pending the outcome of a court case, if the practitioner is acquitted, but the employer feels there is enough evidence to suggest a potential danger to patients, then the Trust has a public duty to act to ensure that the individual concerned does not pose a risk to patient safety.

Similarly, where there are insufficient grounds for bringing charges or the court case is withdrawn there may be grounds for considering police evidence where the allegations would, if proved, constitute misconduct, bearing in mind that the evidence has not been tested in court.

It must be made clear to the police that any evidence they provide and is used in the Trust's case will have to be made available to the doctor or dentist concerned. Where charges are dropped, the presumption is that the employee will be reinstated.

GUIDANCE ON AGREEING TERMS FOR SETTLEMENT ON TERMINATION OF EMPLOYMENT

12. In some circumstances, terms of settlement may be agreed with a doctor or dentist if their employment is terminated. The following good practice principles are set out as guidance:

- Settlement agreements must not be to the detriment of patient safety.

- It is not acceptable to agree any settlement that precludes either appropriate investigations being carried out and reports made or referral to the appropriate regulatory body.
- Payment will not normally be made when a member of staff's employment is terminated on disciplinary grounds or following the resignation of the member of staff.
- Expenditure on termination payments must represent value for money. For example, the Trust should be able to defend the settlement on the basis that it could conclude the matter at less cost than other options. A clear record must be kept, setting out the calculations, assumptions and rationale of all decisions taken, to show that the Trust has taken into account all relevant factors, including legal advice.
- The audit trail must also show that the matter has been considered and approved by the Remuneration Committee and the Board. It must also be able to stand up to district auditor and public scrutiny.
- Offers of compensation, as an inducement to secure the voluntary resignation of an individual, must not be used as an alternative to the disciplinary process.
- All job references must be accurate, realistic and comprehensive and under no circumstance may they be misleading.
- Where a termination settlement is agreed, details may be confirmed in a Deed of Compromise that should set out what each party may say in public or write about the settlement.

The Deed of Compromise is for the protection of each party, but it must not include clauses intended to cover up inappropriate behaviour or inadequate services and should not include the provision of an open reference.

10.4 PART IV**PROCEDURES FOR DEALING WITH ISSUES OF
CAPABILITY****INTRODUCTION & GENERAL PRINCIPLES**

1. The causes of adverse events should not automatically be attributed to the actions, failings or unsafe acts of an individual alone. Root cause analysis of individual adverse events frequently show that these are more broadly based and can be attributed to systems or organisational failures or demonstrate that they are untoward outcomes which could not have been predicted and are not the result of any individual or systems failure. Each will require appropriate investigation and remedial actions.

The National Patient Safety Agency (NPSA) was established to co-ordinate the efforts of all those involved in healthcare to learn from adverse incidents occurring within the NHS. In particular, the NPSA aims to facilitate the development of an open and fair culture, which encourages doctors, dentists and other NHS staff to report adverse incidents and other near misses in a climate free from fear of personal reprimand, where the sharing of experience helps others to learn lessons and in turn improve patient safety.

2. However, there will be occasions where an employer considers that there has been a clear failure by an individual to deliver an adequate standard of care, or standard of management, through lack of knowledge, ability or consistently poor performance. These are described as capability issues. Matters that should be described and dealt with as misconduct issues are covered in Part III of this framework.
3. Concerns about the capability of a doctor or dentist may arise from a single incident or a series of events, reports or poor clinical outcomes. Advice from the National Clinical Assessment Service (Practitioner Performance Advisory Service (PPAS)) will help the Trust to come to a decision on whether the matter raises questions about the practitioner's capability as an individual (health problems, behavioural difficulties or lack of clinical competence) or whether there are other matters that need to be addressed. If the concerns about capability cannot be resolved routinely by management, the matter must be referred to the Practitioner Performance Advisory Service (PPAS) before the matter can be considered by a capability panel (unless the practitioner refused to have his or her case referred). The Trust will involve the Practitioner

Performance Advisory Service (PPAS) in all other cases particularly those involving professional conduct.

4. Matters which may fall under the capability procedures include:
 - Out of date clinical practice.
 - Inappropriate clinical practice arising from a lack of knowledge or skills that puts patients at risk.
 - Incompetent clinical practice.
 - Inability to communicate effectively.
 - Inappropriate delegation of clinical responsibility.
 - Ineffective clinical team working skills.
5. Wherever possible, the aim should be to resolve issues of capability (including clinical competence and health) through ongoing assessment and support. Early identification of problems is essential to reduce the risk of serious harm to patients. The Practitioner Performance Advisory Service (PPAS) has a key role in providing expert advice and support for local action to support the remedial action for a doctor or dentist and should be consulted. A web-based toolkit has been developed and is available at: [www.PRACTITIONER Performance Advisory Service \(PPAS\) .nhs.uk/toolkit](http://www.PRACTITIONER Performance Advisory Service (PPAS) .nhs.uk/toolkit).
6. Any concerns about capability relating to a doctor or dentist in recognised training grades should be considered initially as a training issue and dealt with via the educational supervisor and college or clinical tutor, with close involvement of the Postgraduate Dean from the outset.

How to proceed where conduct and capability issues involved

7. It is inevitable that some cases will cover conduct and capability issues. It is recognised that these cases can be complex and difficult to manage. If a case covers more than one category of problem, they should usually be combined under a capability hearing although there may be occasions where it is necessary to pursue a conduct issue separately.

It is for the Trust to decide on the most appropriate way forward having consulted with an Practitioner Performance Advisory Service (PPAS) adviser and their own employment law specialist.

Duties of Employers

8. The procedures set out below are designed to cover issues where a doctor's or dentist's *capability* to practise is in question. Prior to instigating these procedures, the employer should consider the scope for resolving the issue through counselling or retraining and should take advice from the Practitioner Performance Advisory Service (PPAS).
9. Capability may be affected by ill health. Arrangements for handling concerns about a practitioner's health are described in Part V of this framework. The Trust will follow its own procedure for dealing with ill health – including obtaining advice from the Occupational Health Physician.
10. The Trust will ensure that investigations and capability procedures are conducted in a way that does not discriminate on the grounds of race, gender, disability or indeed on other grounds.
11. The Trust will ensure that managers and case investigators receive appropriate and effective training in the operation of capability procedures. Those undertaking investigations or sitting on capability or appeals panels must have had formal equal opportunities training before undertaking such duties. The Trust Board must agree what training its staff and its members must have completed before they can take a part of these proceedings.

CAPABILITY PROCEDURE

The pre-hearing process

12. When a report of the Trust investigation (as in Part I) has been received, the case manager must give the practitioner the opportunity to comment in writing on the factual content of the report produced by the case investigator. Comments in writing from the practitioner, including any mitigation, must normally be submitted to the case manager within 10 working days from the date of receipt of the request for comments. In exceptional circumstances, for example in complex cases or due to annual leave, the deadline for comments from the practitioner should be extended.
13. The case manager should decide what further action is necessary, taking into account the findings of the report, any comments that the practitioner has made and the advice of the Practitioner Performance Advisory Service (PPAS). The case manager will need to consider urgently:

- Whether action under Part II of the framework is necessary to exclude the practitioner; or
- To place temporary restrictions on their clinical duties.

The case manager will also need to consider with the Medical Director and Director of Human Resources whether the issues of capability can be resolved through local action (such as retraining, counselling, performance review). If this action is not practicable for any reason the matter must be referred to the Practitioner Performance Advisory Service (PPAS) for it to consider whether an assessment should be carried out and to provide assistance in drawing up an action plan. The case manager will inform the practitioner concerned of the decision immediately and normally within 10 working days of receiving the practitioner's comments.

14. The Practitioner Performance Advisory Service (PPAS) will assist the Trust to draw up an action plan designed to enable the practitioner to remedy any lack of capability that has been identified during the assessment.

The Trust must facilitate the agreed action plan (which has to be agreed by the Trust and the practitioner before it can be actioned). There may be occasions when a case has been considered by the Practitioner Performance Advisory Service (PPAS), but the advice of its assessment panel is that the practitioner's performance is so fundamentally flawed that no educational and/or organisational action plan has a realistic chance of success.

In these circumstances, the case manager must make a decision, based upon the completed investigation report and informed by the Practitioner Performance Advisory Service (PPAS) advice, whether the case should be determined under the capability procedure. If so, a panel hearing will be necessary.

15. If the practitioner does not agree to the case being deferred to the Practitioner Performance Advisory Service (PPAS), the panel hearing will normally be necessary.

16. The following procedure should be followed before the hearing:

- The case manager must notify the practitioner in writing of the decision to arrange a capability hearing. This notification should be made at least 20 working days before the hearing and include details of the allegations and the arrangements for proceeding including the practitioner's rights to be accompanied and copies of any documentation and/or evidence that will be made available to the capability panel. This period will give the practitioner sufficient notice to allow them to arrange for a companion to accompany them to the hearing if they so choose.

- All parties must exchange any documentation, including witness statements, on which they wish to rely in the proceedings no later than 10 working days before the hearing. In the event of late evidence being presented, the employer should consider whether a new date should be set for the hearing.
- Should either party request a postponement to the hearing the case manager is responsible for ensuring that a reasonable response is made and that time extensions to the process are kept to a minimum. The Trust retains the right, after a reasonable period (not normally less than 30 working days), to proceed with the hearing in the practitioner's absence, although the Trust will act reasonably in deciding to do so.
- Should the practitioner's ill health prevent the hearing taking place the Trust will implement its usual absence procedures and involve the Occupational Health Department as necessary.
- Witnesses who have made written statements at the inquiry stage may, but will not necessarily, be required to attend the capability hearing. Following representations from either side contesting a witness statement which is to be relied upon in the hearing, the Chairman should invite the witness to attend.

The Chairman cannot require anyone other than an employee to attend. However, if evidence is contested and the witness is unable or unwilling to attend, the panel should reduce the weight given to the evidence as there will not be the opportunity to challenge it properly. A final list of witnesses to be called must be given to both parties not less than two working days in advance of the hearing.

- If witnesses required to attend the hearing choose to be accompanied, the person accompanying them will not be able to participate in the hearing.

The hearing framework

17. The capability hearing will normally be chaired by an Executive Director of the Trust. The panel should comprise a total of 3 people, normally 2 members of the Trust Board, or senior staff appointed by the Board for the purpose of the hearing. At least one member of the panel must be a medical or dental practitioner who is not employed by the Trust. As far as is reasonably possible or practical, no member of the panel or advisers to the panel should have been previously involved in the investigation. In the case of clinical academics, a further panel member may be appointed in accordance with any protocol agreed between the employer and the university.

18. Arrangements must be made for the panel to be advised by:

- A senior member of staff from Human Resources, and
- A senior clinician from the same or similar clinical specialty as the practitioner concerned, but from another NHS employer.
- A representative of a university if provided for in any protocol as mentioned in paragraph 17.

It is important that the panel is aware of the typical standard of competence required of the grade of doctor in question. If for any reason the senior clinician is unable to advise on the appropriate level of competence, a doctor from another NHS employer in the same grade as the practitioner in question should be asked to provide advice.

19. It is for the employer to decide on the membership of the panel. A practitioner may raise an objection to the choice of any panel member within 5 working days of notification. The employer should review the situation and take reasonable measures to ensure that the membership of the panel is acceptable to the practitioner. It may be necessary to postpone the hearing while this matter is resolved. The employer must provide the practitioner with the reasons for reaching its decision in writing before the hearing can take place.

Representation at capability hearings

20. The hearing is not a court of law. Whilst the practitioner should be given every reasonable opportunity to present his or her case, the hearing should not be conducted in a legalistic or excessively formal manner.

21. The practitioner may be represented in the process by a friend, partner or spouse, colleague, or a representative who may be from or retained by a trade union or defence organisation. Such a representative may be legally qualified, but they will not, however, be representing the practitioner formally in a legal capacity. The representative will be entitled to present a case on behalf of the practitioner, address the panel and question the management case and any witness evidence.

Requests to be accompanied by a legal representative will be considered by the Director of HR on a case by case basis. Where the allegations against an employee are serious and dismissal may be an outcome, particularly to the extent that if dismissed the employee may lose any future prospect of employment in his/her profession due to loss or restriction of membership with his/her professional body, it may be considered reasonable to allow an employee legal representation. Requests should be submitted in writing to the

Director of HR and a written response should be given within 5 working days of receipt of the request.

Conduct of the capability hearing

22. The hearing should be conducted as follows:

- The panel and its advisers (see paragraph 18), the practitioner, his or her representative and the case manager will be present at all times during the hearing. Witnesses will be admitted only to give their evidence and answer questions and will then retire.
- The Chairman of the panel will be responsible for the proper conduct of the proceedings. The Chairman should introduce all persons present and announce which witnesses are available to attend the hearing.
- The procedure for dealing with any witnesses attending the hearing shall be the same and shall reflect the following:
- The witness to confirm any written statement and give any supplementary evidence.
- The side calling the witness can question the witness.
- The other side can then question the witness.
- The side which called the witness may seek to clarify any points which have arisen during questioning but may not at this point raise new evidence.

The order of presentation shall be:

- The case manager presents the management case including calling any witnesses. The above procedure for dealing with witnesses shall be undertaken for each witness in turn, at the end of which each witness shall be allowed to leave.
- The Chairman shall invite the case manager to clarify any matters arising from the management case on which the panel requires further clarification.
- The practitioner and/or their representative shall present the practitioner's case, calling any witnesses. The above procedure for dealing with witnesses shall be

undertaken for each witness in turn, at the end of which each witness shall be allowed to leave.

- The Chairman shall invite the practitioner and/or representative to clarify any matters arising from the practitioner's case on which the panel requires further clarification.
- The Chairman shall invite the case manager to make a brief closing statement summarising the key points of the case.
- The Chairman shall invite the practitioner and/or representative to make a brief closing statement summarising the key points of the practitioner's case. Where appropriate this statement may also introduce any grounds for mitigation.
- The panel shall then retire to consider its decision.

Decisions

23. The panel will have the power to make a range of decisions including the following:

- No action required.
- Oral agreement that there must be an improvement in clinical performance within a specified time scale with a written statement of what is required and how it might be achieved. (*stays on employee's record for 6 months*).
- Written warning that there must be an improvement in clinical performance within a specified time scale with a statement of what is required and how it might be achieved. (*stays on employee's record for 1 year*).
- Final written warning that there must be an improvement in clinical performance within a specified time scale with a statement of what is required and how it might be achieved. (*stays on employee's record for 1 year*).
- Termination of contract.

It is also reasonable for the panel to make comments and recommendations on issues other than the competence of the practitioner, where these issues are relevant to the case. For example, there may be matters around the systems and procedures operated by the employer that the panel wishes to comment upon.

24. A record of oral agreements and written warnings should be kept on the practitioner's personal file but should be removed following the specified period.
25. The decision of the panel should be communicated to the parties as soon as possible and normally within 5 working days of the hearing. Because of the complexities of the issues under deliberation and the need for detailed consideration, the parties should not necessarily expect a decision on the day of the hearing.
26. The decision must be confirmed in writing to the practitioner. This notification must include reasons for the decision, clarification of the practitioner's right of appeal and notification of any intent to make a referral to the GMC/GDC or any other external/professional body.

APPEALS PROCEDURES IN CAPABILITY CASES

Introduction

27. Given the significance of the decision of a capability panel to warn or dismiss a practitioner, it is important that a robust appeal procedure is in place. Every Trust must therefore establish an internal appeal process for appeal against decisions of a capability panel. There is no requirement for Trusts to set up a procedure for appeal against exclusion or investigation as these are adjuncts to the stages of the decision making process on what future action to take. The procedure for handling issues about the classification of a case as misconduct is dealt with in Part III of this framework.
28. The appeals procedure provides a mechanism for practitioners who disagree with the outcome of a decision to have an opportunity for the case to be reviewed. The appeal panel will need to establish whether the Trust's procedures have been adhered to and that the panel in arriving at their decision acted fairly and reasonably based on:
 - A fair and thorough investigation of the issue;
 - Sufficient evidence arising from the investigation or assessment on which to base the decision;
 - Whether in the circumstances the decision was fair and reasonable and commensurate with the evidence heard.

It can also hear new evidence submitted by the practitioner and consider whether it might have significantly altered the decision of the original hearing. The appeal panel, however, should not rehear the entire case (see paragraph 30 below).

29. A dismissed practitioner will in all cases be potentially able to take their case to an Employment Tribunal where the reasonableness or otherwise of the Trust's actions will be tested.

The appeal process

30. The predominant purpose of the appeal is to ensure that a fair hearing was given to the original case and a fair and reasonable decision reached by the hearing panel. The appeal panel has the power to confirm or vary the decision made at the capability hearing, or order that the case is reheard. Where it is clear in the course of the appeal hearing that the proper procedures have not been followed and the appeal panel determines that the case needs to be fully reheard, the Chairman of the panel shall have the power to instruct a new capability hearing.
31. Where the appeal is against dismissal, the practitioner should not be paid during the period of appeal, from the date of termination of employment. Should the appeal be upheld, the practitioner should be reinstated and must be paid backdated to the date of termination of employment. Where the decision is to rehear the case, the practitioner should also be reinstated, subject to any conditions or restrictions in place at the time of the original hearing and paid backdated to the date of termination of employment.

The appeal panel

32. The panel will consist of three members. The members of appeal panel must not have had any previous direct involvement in the matters that are the subject of the appeal, for example, they must not have acted as the designated Board member. These members will be:
- An independent member (trained in legal aspects of appeals) from an approved pool. This person is designated Chairman.
 - The Chairman (or other non-executive director) of the employing organisation who must have the appropriate training for hearing an appeal.

- A medically qualified member (or dentally qualified if appropriate) who is not employed by the Trust who must also have the appropriate training for hearing an appeal.
- In the case of clinical academics, a further panel member may be appointed in accordance with any protocol agreed between the employer and the university.

33. The panel should call on others to provide specialist advice. This should normally include:

- A consultant from the same specialty or sub-specialty as the appellant, but from another NHS employer.
- A senior Human Resources specialist.

It is important that the panel is aware of the typical standard of competence required of the grade of doctor in question. If for any reason the senior clinician is unable to advise on the appropriate level of competence, a doctor from another NHS employer in the same grade as the practitioner in question should be asked to provide advice.

34. The Trust should arrange the panel and notify the appellant as soon as possible and in any event within the recommended timetable in paragraph 35. Every effort should be made to ensure that the panel members are acceptable to the appellant. Where in rare cases agreement cannot be reached upon the constitution of the panel, the appellant's objections should be noted carefully. Trusts are reminded of the need to act reasonably at all stages of the process.

35. It is in the interests of all concerned that appeals are heard speedily and as soon as possible after the original capability hearing. The following timetable should apply in all cases:

- Appeal by written statement to be submitted to the Director of Human Resources within 25 working days of the date of the written confirmation of the original decision.
- Hearing to take place within 25 working days of date of lodging appeal.
- Decision reported to the appellant and the Trust within 5 working days of the conclusion of the hearing.

36. The timetable should be agreed between the Trust and the appellant and thereafter varied only by mutual agreement. The case manager should be informed and is responsible for ensuring that extensions are absolutely necessary and kept to a minimum.

Powers of appeal panel

37. The appeal panel has the right to call witnesses of its own volition but must notify both parties at least 10 working days in advance of the hearing and provide them with a written statement from any such witness at the same time.
38. Exceptionally, where during the course of the hearing the appeal panel determines that it needs to hear the evidence of a witness not called by either party, then it shall have the power to adjourn the hearing to allow for a written statement to be obtained from the witness and made available to both parties before the hearing reassembles.
39. If, during the course of the hearing, the appeal panel determines that new evidence needs to be presented, it should consider whether an adjournment is appropriate. Much will depend on the weight of the new evidence and its relevance. The appeal panel has the power to determine whether to consider the new evidence as relevant to the appeal, or whether the case should be reheard, on the basis of the new evidence, by a capability hearing panel.

Conduct of appeal hearing

40. All parties should have all documents, including witness statements, from the previous capability hearing together with any new evidence.
41. The practitioner may be represented in the process by a friend, partner or spouse, colleague or a representative who may be from or retained by a trade union or defence organisation. Such a representative may be legally qualified, but they will not, however, be representing the practitioner formally in a legal capacity. The representative will be entitled to present a case on behalf of the practitioner, address the panel and question the management case and any written evidence.
42. Both parties will present full statements of fact to the appeal panel and will be subject to questioning by either party, as well as the panel. When all the evidence has been presented, both parties shall briefly sum up. At this stage, no new information can be introduced. The appellant (or his/her companion) can at this stage make a statement in mitigation.
43. The panel, after receiving the views of both parties, shall consider and make its decision in private.

Decision

44. The decision of the appeal panel shall be made in writing to the appellant and shall be copied to the Trust's case manager such that it is received within 5 working days of the conclusion of the hearing. The decision of the appeal panel is final and binding. There shall be no correspondence on the decision of the panel, except and unless clarification is required on what has been decided (but not on the merits of the case), in which case it should be sought in writing from the Chairman of the appeal panel.

Action following hearing

45. Records must be kept, including a report detailing the capability issues, the practitioner's defence or mitigation, the action taken and the reasons for it. These records must be kept confidential and retained in accordance with the capability procedure and the Data Protection Act 1998. These records need to be made available to those with a legitimate call upon them, such as the practitioner, the Regulatory Body, or in response to a Direction from an Employment Tribunal.

TERMINATION OF EMPLOYMENT WITH PERFORMANCE ISSUE UNRESOLVED

46. Where the employee leaves employment before disciplinary procedures have been completed, the investigation must be taken to a final conclusion in all cases and capability proceedings must be completed wherever possible, whatever the personal circumstances of the employee concerned.
47. Every reasonable effort must be made to ensure the employee remains involved in the process. If contact with the employee has been lost, the Trust will invite them to attend any hearing by writing to both their last known home address and their registered address (the two will often be the same). The Trust will make a judgement, based on the evidence available, as to whether the allegations about the practitioner's capability are upheld. If the allegations are upheld, the Trust will take appropriate action, such as requesting the issue of an alert letter and referral to the professional regulatory body, referral to the police, or the Protection of Children Act List (held by the Department for Education and Skills).
48. If an excluded employee or an employee facing capability proceedings becomes ill, they should be subject to the Trust's usual sickness absence procedures. The sickness absence procedures take precedence over the capability

procedures and the employer should take reasonable steps to give the employee time to recover and attend any hearing. Where the employee's illness exceeds 4 weeks, they must be referred to the Occupational Health Department. The Occupational Health Department will advise the Trust on the expected duration of the illness and any consequences it may have for the capability process and will also be able to advise on the employee's capacity for future work, as a result of which the Trust may wish to consider retirement on health grounds. Should the employment be terminated as a result of ill health, the investigation should still be taken to a conclusion and the Trust form a judgement as to whether the allegations are upheld.

49. If, in exceptional circumstances, a hearing proceeds in the absence of the practitioner, for reasons of ill health, the practitioner should have the opportunity to submit written submissions and/or have a representative attend in his absence.
50. Where a case involves allegations of abuse against a child, the guidance issued to the NHS in September 2000, called "The Protection of Child Act 1999 – A Practical Guide to the Act for all Organisations Working with Children" gives more detailed information. A copy can be found on the Department of Health website.

Appeal Panels in Capability Cases

51. The framework provides for the appeal panel to be chaired by an independent member from an approved pool trained in legal aspects of appeals.
52. It has been agreed that it would be preferable to continue to appoint appeal panel chairmen through a separately held national list rather than through local selection. The benefits include:
 - The ability to secure consistency of approach through national appointment, selection and training of panel chairmen; and
 - The ability to monitor performance and assure the quality of panellists.
53. The following provides an outline of how it is envisaged that the process will work.

Creating and administering the list

54. The responsibility for recruitment and selection of panel chairs to the list will lie with *the NHS Appointments Commission*. *NHS Employers* will be responsible for administration of the list.
55. Recruitment to the list will be in accordance with published selection criteria drawn up in consultation with stakeholders, including the *BMA*, *BDA* defence organisations, the *Practitioner Performance Advisory Service (PPAS)* and *NHS Employers*. These stakeholders will also assist in drawing up the selection criteria and in seeking nominations to serve.
56. The *Department of Health*, in consultation with *NHS Employers*, the *BDA* and the *BMA* will provide a job description based on the Competence Framework for Chairmen and Members of Tribunals, drawn up by the *Judicial Studies Board*. The framework, which can be adapted to suit particular circumstances sets out six headline competencies features the core elements of law and procedure, equal treatment, communication, conduct of hearing, evidence and decision making. Selection will be based on the extent to which candidates meet the competencies.
57. Panel members will be subject to appraisal against the core competencies and feedback on performance provided by participants in the hearing. This feedback will be taken into account when reviewing the position of the panel member on the list.
58. The level of fees payable to panel members will be set by *NHS Employers* and paid locally by the employing organisation responsible for establishing the panel.
59. List members will be expected to take part in and contribute to local training events from time to time. For example, training based on generic tribunal skills along the lines of the Judicial Studies board competencies and/or seminars designed to provide background on the specific context of NHS disciplinary procedures – including the expectations of employers and representatives, could be provided with support from *NHS Employers*, the *National Clinical Assessment Service* and other stakeholders.

10.5 PART V

HANDLING CONCERNS ABOUT A PRACTITIONER'S HEALTH

INTRODUCTION

1. A wide variety of health problems can have an impact on an individual's clinical performance. These conditions may arise spontaneously or be as a consequence of work place factors such as stress.
2. The principle for dealing with individuals with health problems is that, wherever possible and consistent with reasonable public protection, they should be treated, rehabilitated or re-trained (for example if they cannot undertake exposure prone procedures) and kept in employment, rather than be lost from the NHS.

Retaining the services of individuals with health problems

3. Wherever possible the Trust should attempt to continue to employ the individual provided this does not place patients or colleagues at risk.

Examples of action to take:

- Sick leave for the practitioner (the practitioner to be contacted frequently on a pastoral basis to stop them feeling isolated).
- Remove the practitioner from certain duties.
- Reassign them to a different area of work.
- Arrange re-training or adjustments to their working environment, with appropriate advice from the Practitioner Performance Advisory Service (PPAS) and/or Deanery, under reasonable adjustment provisions in the Disability Discrimination Act 1995.

Reasonable adjustment

3. At all times the practitioner should be supported by their employer and the Occupational Health Department who should ensure that the practitioner is offered every available resource to get back to practise where appropriate. The Trust will consider what reasonable adjustments could be made to their workplace conditions or other arrangements.

Examples of reasonable adjustment:

- Make adjustments to the premises
 - Re-allocate some of the disabled person's duties to another
 - Transfer employee to an existing vacancy
 - Alter employee's working hours or pattern of work
 - Assign employee to a different workplace
 - Allow absence for rehabilitation, assessment or treatment
 - Provide additional training or retraining
 - Acquire/modify equipment
 - Modifying procedures for testing or assessment
 - Provide a reader or interpreter
 - Establish mentoring arrangements
5. In some cases, retirement due to ill health may be necessary. Ill health retirement should be approached in a reasonable and considerate manner, in line with NHS Pensions Agency advice. However, it is important that the issues relating to conduct or capability that have arisen are resolved, using the agreed procedures where appropriate.

Handling health issues

6. Where there is an incident that points to a problem with the practitioner's health, the incident may need to be investigated to determine a health problem. If the report recommends OH Department involvement, the nominated manager must immediately refer the practitioner to a qualified, usually a consultant, occupational physician with the Occupational Health Department.
7. The Practitioner Performance Advisory Service (PPAS) should be approached to offer advice on any situation and at any point where the employer is concerned about a doctor or dentist. Even apparently simple or early concerns should be referred as these are easier to deal with before they escalate.
8. The occupational physician should agree a course of action with the practitioner and send his/her recommendations to the Medical Director and a meeting should be convened with the Director of HR, the Medical Director or case manager, the practitioner and case worker from the OH Department to agree a timetable of action and rehabilitation (where appropriate). The practitioner may wish to bring a support companion to these meetings. This could be a family member, a colleague or a trade union or defence association representative. Confidentiality must be maintained by all parties at all times.

9. If a doctor or dentist's ill health makes them a danger to patients and they do not recognise that or are not prepared to co-operate with measures to protect patients, then exclusion from work must be considered and the professional regulatory body must be informed, irrespective of whether or not they have retired on the grounds of ill health.
10. In those cases where there is impairment of performance solely due to ill health, disciplinary procedures would only be considered in the most exceptional of circumstances, for example if the individual concerned refuses to co-operate with the employer to resolve the underlying situation, e.g. by repeatedly refusing a referral to the Occupational Health Department (OHD) or the Practitioner Performance Advisory Service (PPAS). In these circumstances the procedures in Part IV should be followed.
11. There will be circumstances where an employee who is subject to disciplinary proceedings puts forward a case, on health grounds, that the proceedings should be delayed, modified or terminated. In such cases the employer is expected to refer the doctor or dentist to the OHD for assessment as soon as possible. Unreasonable refusal to accept a referral to, or to co-operate with, the OHD under these circumstances, may give separate grounds for pursuing disciplinary action.
12. Special Professional Panels (generally referred to as the "Three Wise Men") were set up by District Health Authorities under circular HC(82)13. This responsibility was not transferred to Trusts and the process has fallen into disuse in most parts of the country. This part of the framework replaces HC(82)13 which is cancelled, and any existing panels have been disbanded.

APPENDIX 1

DECISION MAKING GROUP

TERMS OF REFERENCE

Name	KGH Decision Making Group (DMG)
Function	To ascertain if there is a concern with regards to a Medical practitioner as far as capability/conduct is concerned
Purpose 1. To undertake on behalf of the Trust a screening process once a concern has been raised to determine if an investigation or an intervention is required 2. Soft concerns and complaints resulting in patient harm can also be taken through the same screening process 3. DMG can be called to meet on an ad hoc basis by MD/ DMD or Director of HR 4. DMG can seek expert opinion locally to clarify if there is a case to answer 5. DMG should be fully supported to access clinical records, recent SIRIs, complaints against the Medical practitioner in order to come to the decision 6. DMG will be responsible to contact Practitioner Performance Advisory Service (PPAS) as and when necessary 7. DMG will also discuss the need to inform other regulators including GMC 8. If there are grounds for investigation, DMG should set out TOR for the investigation, appoint an investigator 9. DMG should designate an individual from the group to keep the Medical practitioner informed 10. DMG will be responsible for appointing case investigator, case manager and timelines 11. DMG will meet again (frequency to be determined in first meeting) to update on progress if required 12. DMG will be called to meet as and when the need arises and the quorate will be	
Decisions/Recommends/Authority	Advisory
Financial Authority	Nil
Reports to	
Reports required to help arrive at decision	Patient Notes SIRIs Complaints

Frequency and Duration	Ad hoc and update meeting if relevant
Chair	HR Director/ Medical Director
Vice Chair	DMD/CHIEF OF DIVISION
Minute taker/meeting co-ordinator	PA to HR Director

Quorum	3 members*
Membership	Medical Director Deputy MD HR Director/ Deputy Chief of Division concerning practitioner in question and other Chief of Division not directly involved Chief Operating Officer

Endorsed by: Workforce Development Committee